

METROCARE DIABETES CENTER

DISCHARGE SUMMARY

DEPARTMENT OF ENDOCRINOLOGY

Patient Details

General Information

UHID: MDC/ENDO/2026/1443

IP No: IP/2610/0418

Patient Identifier

Ward/Bed Number: DAY CARE – D03

Name: **MR. ROHAN MEHTA**

Address: Velachery, Chennai, Tamil Nadu, INDIA

Age/Sex: 53 / Male

Treating Consultants

Primary Consultant: **Dr. Vikram Sethi**

Consultant: DR. VIKRAM SETHI, MD (ENDOCRINOLOGY)

Policy Details: Self-Paid

Date of Admission: 05/10/2026 – 09:30 AM

Date of Discharge: 07/10/2026 – 02:00 PM

Diagnosis

Type 2 Diabetes Mellitus – Moderately Controlled
Chronic Kidney Disease – Stage 3a
Diabetic Nephropathy

Chief Complaints

- Persistent pedal edema
 - Occasional breathlessness on exertion
 - Fatigue
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History of Present Illness

Known case of Type 2 Diabetes Mellitus and recently diagnosed Chronic Kidney Disease Stage 3a. Patient presented for follow-up due to persistent swelling of feet and rising creatinine levels noted in recent outpatient labs.

Home blood sugar readings averaging 140–180 mg/dL fasting.

Past History

- Type 2 Diabetes Mellitus (8 months)
 - CKD Stage 3a
 - Hypertension (6 years)
 - No known drug allergies
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Clinical Examination

General Examination

Pallor: Mild
Icterus: No
Cyanosis: No
Clubbing: No
Edema: Bilateral Pedal Edema +

Temperature: 98.4°F
Pulse: 86 /min
BP: 158/94 mmHg
RR: 20 /min
SpO₂: 97% on Room Air

Weight: 84 kg

Systemic Examination

CVS: S1 S2 +
RS: NVBS +, No crepitations
P/A: Soft
CNS: Conscious, Oriented

Investigation Reports

Serum Creatinine: 2.0 mg/dL
Blood Urea: 54 mg/dL
eGFR: 44 mL/min/1.73m²
Urine Albumin-Creatinine Ratio: 210 mg/g
HbA1c: 7.6%
Potassium: 4.9 mmol/L
Hemoglobin: 10.5 g/dL

ECG: Normal sinus rhythm

Course in the Hospital

Patient admitted for renal and glycemic optimization. Rising creatinine suggests progression of diabetic nephropathy within CKD Stage 3 spectrum.

Metformin discontinued due to declining renal function. Glycemic control shifted toward renal-safe antidiabetic therapy. Blood pressure regimen intensified.

Nephrology co-management recommended.

Treatment Given During Hospital Stay

TAB. SITAGLIPTIN 50mg – 1-0-0
TAB. EMPAGLIFLOZIN 10mg – 1-0-0
TAB. TELMISARTAN 40mg – 1-0-1
TAB. ATORVASTATIN 10mg – 0-0-1
TAB. TORSEMIDE 10mg – 1-0-0

Low salt + Renal diet

Condition on Discharge

Vitals Stable: Yes
Edema: Mildly Reduced
Renal Function: Impaired but stable

Advice on Discharge

- Strict blood pressure monitoring
 - Renal diet (Low sodium, moderate protein restriction)
 - Daily weight check
 - Avoid nephrotoxic drugs
 - Monitor blood sugars regularly
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Follow-up

Review after 2 weeks with RFT, electrolytes, HbA1c.
Nephrology appointment scheduled.

For Appointment Contact: +91 44 4555 9080

Dr. VIKRAM SETHI

Consultant – Endocrinology

(Signature)

Prepared By: Medical Records Department

MetroCare Diabetes Center
Chennai